

PT-141

MC3R / MC4R Agonist | 10mg x 10 Vials

Sexual Function Research | MC4R Activation | SubQ Injection

Purity: 99.8% | Endotoxin-Free | Research Use Only

PT-141 (Bremelanotide) is a synthetic cyclic heptapeptide derived from Melanotan II, developed specifically for sexual dysfunction research. It acts centrally via MC3R and MC4R (not peripheral vascular mechanisms), producing sexual arousal independently of the nitric oxide pathway. FDA-approved as Vyleesi for hypoactive sexual desire disorder (HSDD) in premenopausal women.

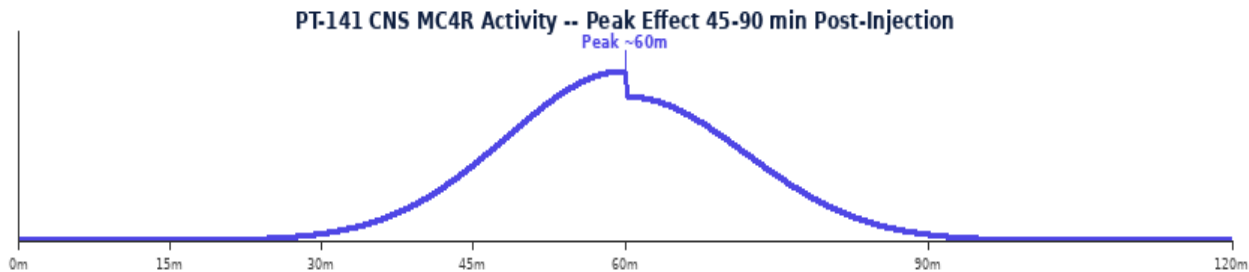
PT-141 Vial Comparison



10mg
10.0mg/mL

Parameter	Specification
Class	Cyclic melanocortin peptide -- MC3R/MC4R agonist (Bremelanotide)
Vial Size	10mg x 10 vials Purity 99.8%
Reconstitution	1 mL BAC Water -> 10 mg/mL (10,000 mcg/mL)
Standard Dose	500-2,000 mcg (0.5-2mg) per injection SubQ
Timing	Inject 45-90 min before activity no fasting required
FDA Status	Approved as Vyleesi (1.75mg) for HSDD in premenopausal women
Cycle & Storage	Use as needed (not daily) 2-8 deg C use within 28 days

MECHANISM OF ACTION & RESEARCH BENEFITS



Pathway	Mechanism	Benefit
MC4R Central	Binds MC4R in hypothalamus (paraventricular nucleus); activates dopaminergic/oxytocin pathways; central arousal signalling independent of testosterone or nitric oxide	Sexual arousal; HSDD research; erectile function
MC3R Metabolic	MC3R activation contributes to energy balance modulation alongside sexual function; mild appetite effects at high doses	Complementary metabolic research applications
No Vascular Mechanism	Does NOT act via PDE5 inhibition or nitric oxide pathway; central (CNS) mechanism of action; works in patients unresponsive to PDE5 inhibitors; different indication profile	Alternative research model to PDE5 inhibitors; unique CNS pathway

Benefit	Context
FDA-Validated Mechanism	Bremelanotide (Vyleesi) is FDA-approved -- the MC4R central arousal pathway is clinically validated for HSDD treatment.
Central Not Peripheral	Unlike PDE5 inhibitors, PT-141 acts centrally via hypothalamic MC4R -- works in patients with neurogenic dysfunction.
Both Sexes	Research applications in both male and female sexual dysfunction via the same central melanocortin pathway.

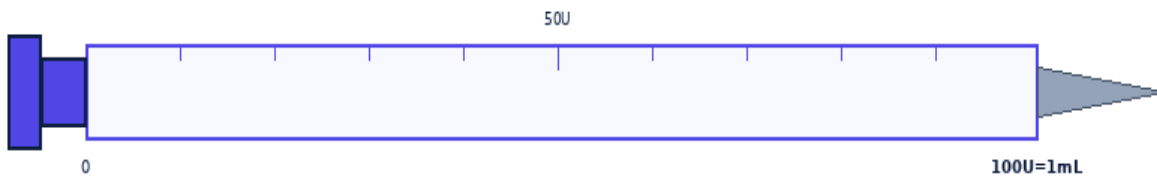
RECONSTITUTION & DOSE CALCULATION

All vials use 1 mL BAC Water. Room temperature. Swirl gently -- never shake.



1 mL BAC Water per vial. Roll gently -- NEVER shake.

Step	Action	Detail
1	Gather	Vial, BAC Water, insulin syringe (29-31G), swabs, sharps.
2	Clean stoppers	Wipe both stoppers. Air-dry 10-15 sec.
3	Draw 1mL BAC	Exactly 1 mL. Expel air bubbles.
4	Inject into vial	Aim at GLASS WALL. Slowly. Never onto powder.
5	Swirl & label	Roll 20-30 sec. Clear within 60 sec. Never shake. Label date. 2-8 deg C. Use within 28 days.



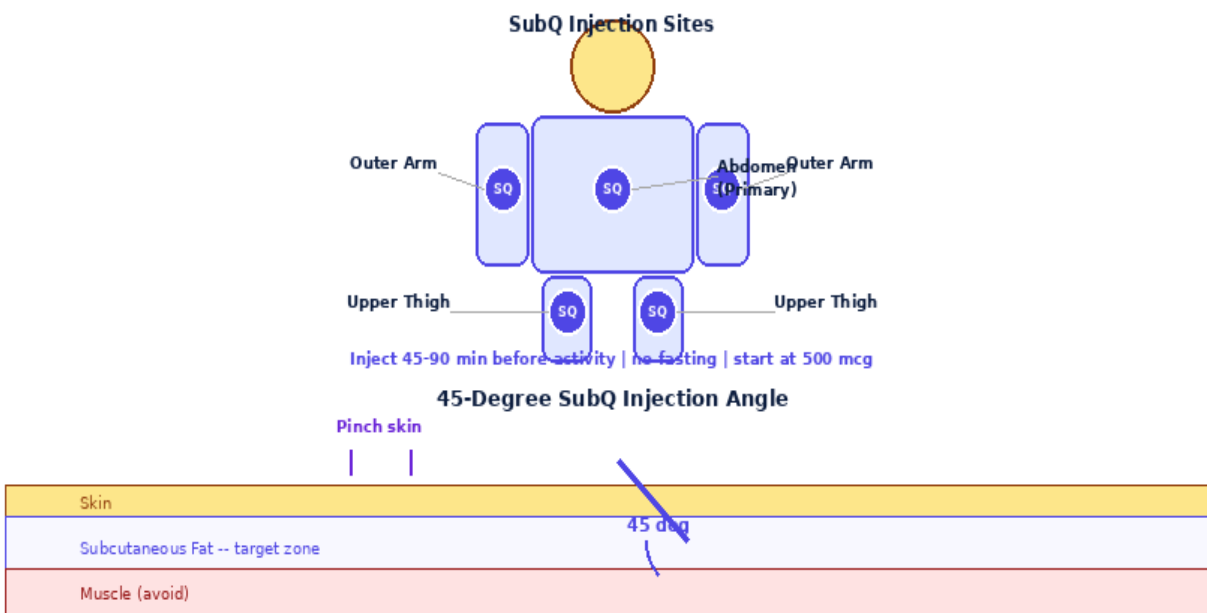
1mL Insulin Syringe | 29-31G | SubQ

Formula: $\text{Vol(mL)} = \text{Dose(mcg)} / \text{Conc(mcg/mL)}$ | **Units:** $\text{Vol} \times 100$

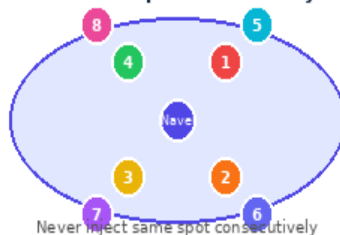
10mg x 10 -- 1mL BAC -> 10.0mg/mL (10,000mcg/mL)

Dose(mcg)	500mcg	750mcg	1000mcg	1500mcg	2000mcg
Vol(mL)	0.0500	0.0750	0.1000	0.1500	0.2000
Units	5.0U	7.5U	10.0U	15.0U	20.0U
Doses/Vial	20	13	10	6	5

INJECTION TECHNIQUE, PROTOCOLS & GOLDEN RULES



8-Point Rotation Map -- Rotate Every Injection



Step	Action	Detail
1	Timing	Inject 45-90 min before desired activity for peak effect.
2	Draw dose	Draw exact volume. Standard research dose: 1,000-1,750 mcg.
3	Inject SubQ	Abdomen preferred. 45 deg. Pinch skin. Inject slowly.
4	Monitor	Nausea common 30-60 min post-injection. Rotate sites.

Protocol	Dose	Frequency	Duration	Notes
Research Dose	1,000-1,750 mcg	As needed (not daily)	Per occasion	45-90 min before activity. Max 1x per 24hr.

Side Effect	Likelihood	Management
Nausea	Very Common	Most common side effect. Start at 500 mcg. Anti-nausea pre-treatment helps.
Flushing	Common	Transient vasodilation. Resolves 30-60 min.
Transient BP increase	Common	Monitor in hypertensive subjects. Contraindicated in uncontrolled hypertension.
Headache	Occasional	Dose-related. Reduce dose.

Contraindications: Cancer history, pregnancy, under 18.

1. 1 mL BAC Water -> 10mg/mL. FDA clinical dose is 1.75mg (175 Units on 1mL syringe).
2. Inject 45-90 minutes before desired activity for optimal timing.
3. No fasting required. Inject any time.
4. Start at 500-750 mcg to assess nausea tolerance before escalating.
5. Nausea is the most common side effect -- antiemetic 30 min prior helps.
6. Do not use more than once per 24-hour period.
7. Contraindicated in uncontrolled hypertension and cardiovascular disease.
8. Rotate SubQ sites. Store 2-8 deg C, use within 28 days.
9. Not for use during pregnancy, lactation, or in individuals under 18.